

Migraine Pain Diary

A neurologist-ready migraine record for phases, aura, trigger stacking, treatment response, and the days between attacks.

Routing Note

- Need:**

Migraine is a sequence, not just an event. This sheet is built to catch the phases before they blur into one word: headache.
- Tool:**

Use it for neurology, aura mapping, trigger stacking, menstrual migraine review, or medication-overuse questions.
- Rule:**

Track the clear days too. Relief days prove whether treatment is doing anything real.

Name: _____

Date Range: _____

Pain Scale Reference (0–10 NRS)



Migraine Episode Log

Record each migraine episode. Document onset, peak, resolution, and total duration. Mark location: L = Left, R = Right, B = Bilateral.

Date	Start	Peak	End	Pain 0-10 (onset)	Pain 0-10 (peak)	Side (L/R/B)	Quality (throb/press/stab)

Headache Location Map

For each episode, mark the primary pain location(s): Frontal, Temple (L/R), Behind eye (L/R), Top of head, Occipital (back), Neck.

Date	Frontal	L Temple	R Temple	L Eye	R Eye	Top	Occipital	Neck

Prodrome Symptoms (Before the Headache)

Check any symptoms noticed 1–48 hours before the headache started. These early warning signs help predict attacks.

Date	Mood Change	Food Cravings	Yawning	Neck Stiffness	Fatigue	Irritability	Concentration Difficulty

Aura Tracking

If aura occurs, log the type, onset time, duration, and description. Most auras last 5–60 minutes and precede the headache.

Date	Type (Visual/Sensory /Speech)	Start Time	Duration (min)	Description (zigzags, spots, tingling, etc.)

Associated Symptoms During Attack

Rate severity: 0 = None, 1 = Mild, 2 = Moderate, 3 = Severe. These help classify migraine type.

Date	Light Sens.	Sound Sens.	Smell Sens.	Nausea	Vomiting	Dizziness	Vision Changes	Neck Pain

Daily Trigger Diary — Dietary & Hydration

Track daily even on migraine-free days. Check if present. Most triggers have a 12–48 hour delay. Compare trigger days vs. non-trigger days.

Date	Aged Cheese	Alcohol (type)	Caffeine (cups)	Processed Meats	Chocolate	Artificial Sweetener	Skipped Meal	Water (glasses)

Daily Trigger Diary — Environmental & Lifestyle

Check any factors present each day. Add your own triggers in the blank columns. Look for combinations — most attacks need 2+ triggers.

Date	Bright Lights	Strong Smells	Weather Change	Stress (0-10)	Screen Time (hrs)	Exercise	_____	_____

Sleep & Hormonal Tracking

Sleep disruption is the #1 modifiable migraine trigger. Track consistently. Hormonal cycle day is essential for menstrual migraine patterns.

Date	Bedtime	Wake Time	Hours Slept	Sleep Quality (1-5)	Cycle Day	Hormone Meds?	Stress Level (0-10)

Date	Bedtime	Wake Time	Hours Slept	Sleep Quality (1-5)	Cycle Day	Hormone Meds?	Stress Level (0-10)

Acute Medication Log

Record every medication taken for an active migraine. Track time-to-relief precisely — this guides treatment optimization. Note: >10 triptan days/month or >15 NSAID days/month risks medication overuse headache (MOH).

Date	Medication Name	Dose	Time Taken	Minutes to Relief	Relief % (0-100)	Side Effects	Notes

Preventive Medication Tracking

Track daily adherence to preventive medications (beta-blockers, anticonvulsants, antidepressants, CGRP inhibitors, etc.).

Date	Med 1: _____	Med 2: _____	Med 3: _____	Taken On Time?	Side Effects Today

Non-Drug Treatments & Self-Care

Track non-medication interventions. Rate effectiveness 0–10. This helps identify what works best for you.

Date	Dark Room / Rest	Ice / Cold Pack	Heat	Caffeine	Relaxation / Breathing	Other: _____	Effective? (0-10)

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Monthly Summary — Bring This to Your Appointment

Complete at month-end. This single page gives your neurologist the complete clinical picture. Fill in totals and observations.

Month / Year: _____

Total Migraine Days This Month: _____

Total Headache-Free Days: _____

Average Pain Intensity (0–10): _____

Worst Single Attack Intensity: _____

Longest Attack Duration (hours): _____

Acute Medication Days This Month: _____

Top Triggers Identified This Month:

1. _____
2. _____
3. _____
4. _____
5. _____

Most Effective Treatments:

1. _____
2. _____
3. _____

Functional Impact:

Work/School Days Missed: _____

Social Events Cancelled: _____

Days with Reduced Function: _____

Questions for My Neurologist

Doctor's Notes / Plan Changes

Quick Reference: Migraine Phases

Phase	Timing	Common Symptoms	Track These
Prodrome	1–2 days before	Mood changes, cravings, yawning, neck stiffness, fatigue	Earliest warning signs — can predict attack
Aura	5–60 min before/during	Visual (zigzags, spots), tingling, numbness, speech difficulty	Type, duration, progression pattern
Headache	4–72 hours	Throbbing pain, photo/phonophobia, nausea, vomiting	Intensity, location, quality, timeline
Postdrome	24–48 hours after	Brain fog, fatigue, weakness, mood changes, residual sensitivity	Duration, severity — counts as disability time

Common Trigger Categories

Category	Top Triggers
Dietary	Aged cheese, red wine, processed meats, chocolate, artificial sweeteners, MSG, caffeine withdrawal, citrus
Environmental	Bright/flickering lights, strong smells, barometric pressure changes, high altitude, loud noise, screen glare
Lifestyle	Sleep changes, skipped meals, dehydration, intense exercise, irregular schedule, travel, poor posture, teeth clenching
Hormonal	Menstruation, perimenopause, oral contraceptives, hormone therapy, ovulation
Emotional	Acute stress, stress letdown (weekend migraine), anxiety, emotional shock, post-event relaxation

When to Seek Emergency Care

- "Worst headache of my life" — sudden, severe onset (thunderclap headache)
- Headache with fever, stiff neck, rash, confusion, or seizures
- Headache after head injury, especially with worsening symptoms
- New neurological symptoms: weakness, vision loss, difficulty speaking, or walking
- Migraine lasting >72 hours (status migrainosus) — call your neurologist
- Aura lasting >60 minutes or aura without headache for the first time

Migraine Rules

Track the quiet days too. Migraine logic usually shows up in the contrast, not just the attack.
Prodrome notes are an early warning record, not trivia. Catch them when they surface.
Bring the summary page when you need the neurologist to see the system fast.